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Linh Tran

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EDUCATION

Johns Hopkins Bloomberg School of Public Health 2023–2027 *(expected)*
PhD Candidate, Health Economics and Policy
Advisors: Matt Eisenberg and Karen Shen
Dissertation: Large Online Mental Health Practices and Demand and Supply of Mental Health Services
Committee Members: Matt Eisenberg, Karen Shen, Sachini Bandara, Rheanna Platt

Princeton University 2020
BA, Economics, *summa cum laude*

RESEARCH INTERESTS

Fields: applied microeconomics, health economics

Interests: mental health services, provider behavior, Medicaid

JOB MARKET PAPER

"Digital Platforms and Provider Behavior: Evidence from Mental Health Services"

Many mental health providers do not participate in insurance due to administrative burden, increasing costs for patients and contributing to unmet need for care. A new type of digital platform has emerged to alleviate this market friction: large online mental health practices (hereafter "LOMPs") acting as large group practices that organize providers under a single group practice, contract with payers on their behalf, and provide a convenient platform for patients to search and match with in-network providers. Over 30% of therapists in Colorado used an LOMP in 2023. These platforms may increase access to insurance-paid care for patients, but may also increase consolidation and reallocate care to certain patients over others. Using the Colorado All Payer Claims Database, I study the impact of these platforms on providers' patient mix and on spending. I find that providers increased their share of commercial patients by 20.3 percentage points and lowered their share of Medicaid patients by 17.9 percentage points upon joining an LOMP. I find that monthly reimbursements per patient increased by $36, attributable to providers delivering longer sessions and receiving higher payments per procedure code. These findings highlight the potential for these platforms to attract providers away from publicly insured patients and raise concerns of higher spending.

PUBLICATIONS

Coleman Drake, Linh Tran, and Matthew Eisenberg. Medical Cannabis Certifications After Pennsylvania Added Anxiety Disorders as a Qualifying Condition. *Annals of Internal Medicine*, 178(8):1209–1211, August 2025.

We study the impact of adding anxiety disorders as a qualifying condition for medical cannabis certifications in Pennsylvania on the distribution of qualifying conditions.

We find that the share and number of medical cannabis certifications with anxiety disorders as a qualifying condition rapidly increased upon this policy change.

UNDER REVIEW

"A Substantial Share of Medicare Patients Likely Pay Cash for Psychiatric Care: Findings from A Novel Algorithm Using Claims" (first-author, with Karen Shen and Mark Meiselbach, revise and resubmit at Health Services Research)

Low insurance acceptance rates among psychiatrists may require patients to pay fully out-of-pocket ("cash pay") for care, potentially imposing financial burden and deterring treatment. Yet, research quantifying the prevalence of cash pay among psychiatry patients and describing the characteristics of those patients is limited. Using Medicare claims for a random 20% sample of traditional Medicare beneficiaries prescribed mental health medications by a psychiatrist or primary care physician (PCP) between 2016 and 2020, we develop a novel algorithm to identify patients likely paying cash for psychiatrist services and describe their demographic and clinical characteristics. 15.2% of Medicare beneficiaries who saw a psychiatrist likely paid cash for their visits, compared to 2.5% of PCP patients receiving similar medications. Patients who likely paid cash for psychiatrist services were more likely to reside in high-income areas and had prescriptions and diagnoses consistent with less serious mental illnesses. The prevalence of paying cash for psychiatrist services varied more than 7-fold across metropolitan areas. The findings underscore potential inequities in care allocation and highlight the need for ongoing policy efforts to improve insurance acceptance among psychiatrists.

WORKS IN PROGRESS

"Corporatization in Inpatient Psychiatry and Its Implications for Cost and Quality of Care" (with Karen Shen and Adam Sacarny)

Inpatient psychiatric care is increasingly moving away from psychiatric units in general acute care hospitals toward freestanding hospitals, which are increasingly operated by large for-profit chains. We examine the impact of this phenomenon by studying how openings of chain-operated psychiatric hospitals affect access to inpatient psychiatric care and patient outcomes using Traditional Medicare claims and a difference-in-differences approach. Preliminary results suggest that openings of new psychiatric hospitals are associated with higher supply of psychiatric beds, higher mental health-related inpatient admissions to freestanding psychiatric hospitals, and lower admissions to psychiatric units and general hospitals, suggesting a substitution effect among these facilities.

"Characteristics of Patients Using Crisis Care Services in Arizona" (with Karen Shen and Matt Eisenberg)

As mental health crises become increasingly common and contribute to overcrowding of emergency departments (ED), states are expanding mental health crisis care as a community-based and cost-effective alternative to the ED. Understanding utilization patterns and outcomes for patients receiving care at crisis care centers versus the

ED provides crucial evidence as policymakers consider funding these services. Using Medicaid claims from the 2021 Transformed Medicaid Statistical Information System Analytic Files, we explore how characteristics of people using crisis care differ from those using the emergency department for mental health-related reasons in Arizona. We find that crisis care is disproportionately used by patients with serious mental illness (bipolar disorder or schizophrenia), suggesting that crisis centers may serve as an effective alternative to the ED for people with chronic conditions who may otherwise be frequent ED utilizers.

"Impact of Medicaid Expansion on Transport Refusals after Nonfatal Overdoses: A National Analysis of EMS Data" (with Jirka Taylor, Alexander McCourt, and Karen Shen)

Many people who experience a nonfatal overdose and call emergency medical services ("EMS") refuse transportation to the emergency department for further care. One possible reason is concerns about the costs of emergency care, which may deter patients from seeking care and lead to higher risk of adverse events. Using 2018-2024 data from the National Emergency Medical Services Information System, we examine whether increased insurance coverage through Medicaid expansion affected rates of transport refusal rates for nonfatal overdoses involving EMS. We use staggered difference-in-differences to compare trends in refusals between states that expanded Medicaid during the study period versus states that have yet to expand Medicaid. Preliminary evidence suggests that Medicaid expansion is associated with a reduction in transport refusal rates.

TEACHING EXPERIENCE

Johns Hopkins University

Graduate Teaching Assistant

Applied Microeconomics for Policymaking[†]	Fall 2024
Health Economics[†]	Fall 2024, Fall 2025
Introduction to Health Economics[†]	Spring 2025
Econometric Methods for the Evaluation of Health Programs[†]	Spring 2026

Princeton University

Peer Tutor	Spring 2017–Spring 2019
Undergraduate Course Assistant	Fall 2017–Spring 2020

[†]Graduate-level

CONFERENCE PRESENTATIONS

2026: "Estimating the Size and Characteristics of the Cash-Pay Market for Psychiatry in Claims Data", ASHEcon Annual Conference

SERVICE

Academic Policy and Admissions Committee	2023–2025

OTHER EMPLOYMENT

Quantitative Economic Solutions, LLC 2021–2023
Associate Consultant

Stanford Law School 2020–2021
Research Fellow to Dr. Alison Morantz

HONORS AND FELLOWSHIP

Princeton University

Summer Research Grant 2019

Phi Beta Kappa 2020